

Estimating Nonlinear Insulin-Glucose Dynamics in Critically-ill Patients

Markus Mulvihill

KTH Royal Institute of Technology

September, 2026



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Thesis Presentation

Estimating Nonlinear Insulin-Glucose Dynamics in Critically-ill Patients

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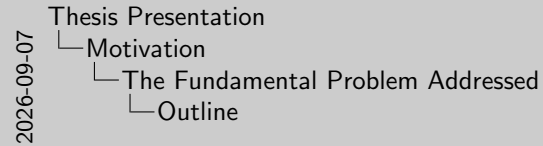
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1. Supervisor - Martin Lindström
2. Examiner - Ragnar Thobaben
3. Opponent - Divyayan Day

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 - The Fundamental Problem Addressed
 - Background
 - Previous Work
- 2 Results and Contributions
 - Main Results
 - Key Ideas Behind Implementation
- 3 Summary
 - Takeaways
 - Further Reading



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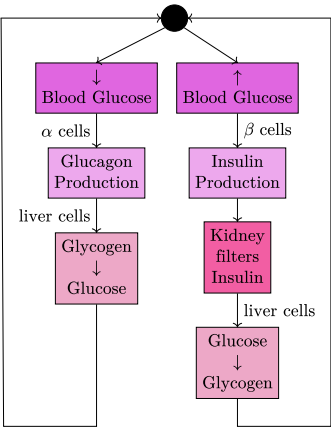
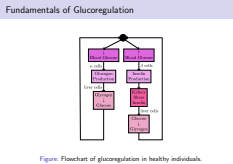


Figure: Flowchart of glucoregulation in healthy individuals.

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 - Motivation
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 - Fundamentals of Glucoregulation



1. The thesis revolves around the functionality of glucoregulation.
2. A high level diagram of the human insulin-glucose system can be described as the image.

Insulin Resistance in ICU Patients

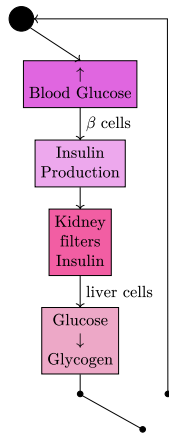


Figure: Consequence of insulin resistance in ICU patients.

- Stress-induced insulin resistance has been observed in critically ill patients.
- Impaired glycogen production in the liver creates an excess of blood glucose (hyperglycaemia).

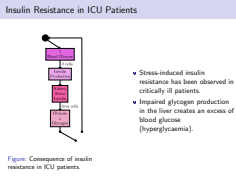
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Motivation

The Fundamental Problem Addressed

Insulin Resistance in ICU Patients



Mention that the break in the chain is just for visual purposes.

Intensive Insulin Therapy

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- Motivation

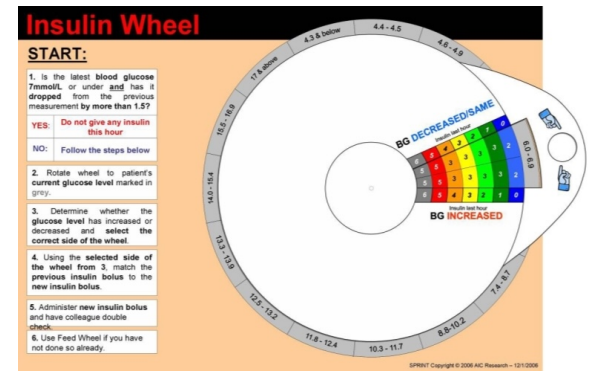
-The Fundamental Problem Addressed

-Intensive Insulin Therapy

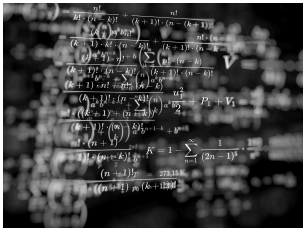
- **What:** Utilizes aggressive insulin infusions and feeding inputs to tightly control blood glucose.
- **Goal:** Prevents hyperglycaemia in patients to reduce **mortalities** and longer visitation stays.
- **How:** Traditionally has relied on ad-hoc sliding protocols.

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- **How:** Traditionally has relied on ad-hoc sliding protocols.

Overview of Ad-hoc Sliding Protocols



Role of Model-based Glycaemic Control



Physiological models can provide more insight into patients' insulin-glucose dynamics than solely relying on glycaemic measurements.

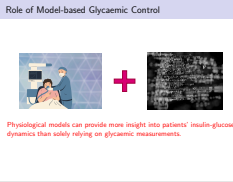
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└ Motivation

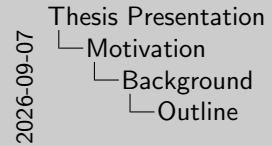
└ The Fundamental Problem Addressed

└ Role of Model-based Glycaemic Control



Insulin infusions combined with better understanding of dynamics can provide more insight to the clinicians

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Intensive Control Insulin-Nutrition-Glucose (ICING) model

Lin et al. has developed a clinically-validated nonlinear insulin-glucose model with the following main components

Dynamic	Notation	Value
Blood Glucose	$\dot{G}$	mmol/L
Interstitial Plasma	$\dot{Q}$	mU/L
Plasma Insulin	$\dot{I}$	mU/L
Glucose in the Stomach	$\dot{P}_1$	mmol
Glucose in the Gut	$\dot{P}_2$	mmol
Endogenous Insulin Production	$u_{en}(t)$	mU/min
Insulin Sensitivity	$S_I(t)$	-
Input	Notation	Value
Exogenous Insulin	$u_{ex}(t)$	mU/min
External Food	$D(t)$	mmol/min
Parenteral Glucose	$P_N(t)$	mmol/min

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More details on the exact formulation of the dynamics can be read in the paper in further reading.

Computational Approximation of the ICING model

- The ICING model equations are defined in continuous-time
- Discretization of the model is required for the presented estimation methods

For an example dynamic F

$$\begin{aligned}t_k &= t_{k-1} + \Delta, \\ \dot{F} &\approx \frac{F(t_k) - F(t_k - \Delta)}{\Delta}, \\ F_k &\approx F_{k-1} + \Delta \dot{F}.\end{aligned}$$

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1. According to Euler's method
2. Discretization error is bounded by Δ

The discrete-time dynamic ICING model can be represented in the stochastic state-space.

State-space Representation

$$x_k \sim p(x_k \mid x_{k-1}, u_{k-1})$$
$$y_k \sim p(y_k \mid x_k)$$

Parameter Description

System Variable	Notation
Dynamics	x_k
Inputs	u_k
Measurements	y_k

There are various methods of optimal filtering, and each has their own assumptions and limitations

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- └ Bayesian Filtering

Bayesian Filtering

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System Variable	Notation
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There are various methods of optimal filtering, and each has their own assumptions and limitations

- 1. The measurement often depends on the hospital, but will be discussed more later
- 2. Just the EKF and the PF will be in the topic of this thesis

Extended Kalman Filter

Approximates **nonlinear** and **Gaussian** dynamic and measurement models as a linear transformation.

1 Prediction step

$$m_k^- = f(m_{k-1}, u_{k-1}),$$

$$P_k^- = F_x(m_{k-1})P_{k-1}F_x^T(m_{k-1}) + F_q(m_{k-1})Q_{k-1}F_q^T(m_{k-1}).$$

2 Update step

$$v_k = y_k - h(m_k^-),$$

$$S_k = H_x(m_k^-)P_k^-H_x^T(m_k^-) + H_r(m_k^-)R_kH_r^T(m_k^-),$$

$$K_k = P_k^-H_k^T(m_k^-)S_k^{-1},$$

$$m_k = m_k^- + K_k v_k,$$

$$P_k = P_k^- - K_k S_k K_k^T.$$

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$$m_k = m_k^- + K_k v_k,$$

$$P_k = P_k^- - K_k S_k K_k^T.$$

Bootstrap Particle Filter

Formulates the posterior of the dynamic function as a Monte Carlo approximation.

- 1 Sample N particles from the dynamic function

$$x_k^{(i)} \sim p(x_k | x_{k-1}^{(i)}, u_{k-1}) \quad i = 1, \dots, N.$$

- 2 Update weights from the measurement

$$w_k^{*(i)} \propto p(y_k | x_k^{(i)}) \quad i = 1, \dots, N,$$

$$w_k^{(i)} = \frac{w_k^{*(i)}}{\sum_{j=1}^N w_k^{*(j)}} \quad i = 1, \dots, N.$$

- 3 Systematically resample particles from a distribution proportional to the weights

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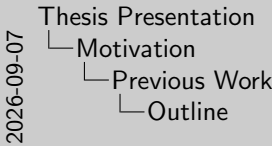
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Formulation of Synthetic Patients

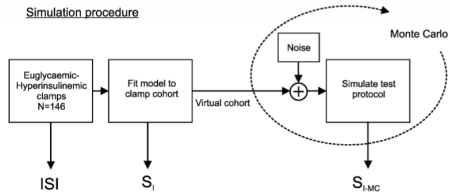


Figure: An example of a simulation procedure (Lotz et al. 2008).

- Synthetic patient trajectories model clinical insulin–glucose dynamics.
- Existing methods typically perturb previously recorded clinical data.

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Formulation of Synthetic Patients

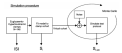


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Estimation of Patient Insulin-Glucose Dynamics

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Motivation

Previous Work

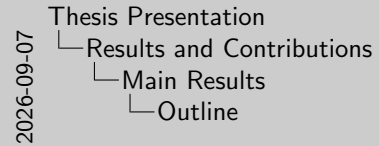
Estimation of Patient Insulin-Glucose Dynamics

- Previous studies have employed Bayesian filtering methods:
 - Extended and unscented Kalman filters
 - Particle filters
- Practical implementation is often insufficiently addressed:
 - Constant nutrition and insulin inputs
 - Known initial insulin-glucose dynamics
- Estimator configurations and performance are not systematically compared.

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Generating Synthetic Patient Trajectories

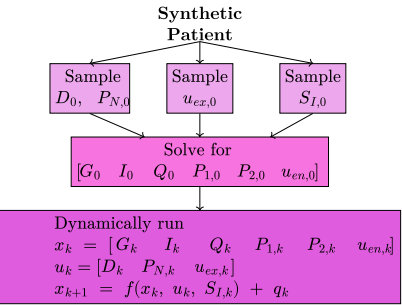


Figure: Proposed framework.

- Sampled patient inputs from observed distributions and insulin sensitivity from a predefined distribution.
- Generated diverse initial insulin–glucose states across synthetic patients.
- Simulated unique trajectories over time without manual interventions.

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Synthetic Patients' Distributions

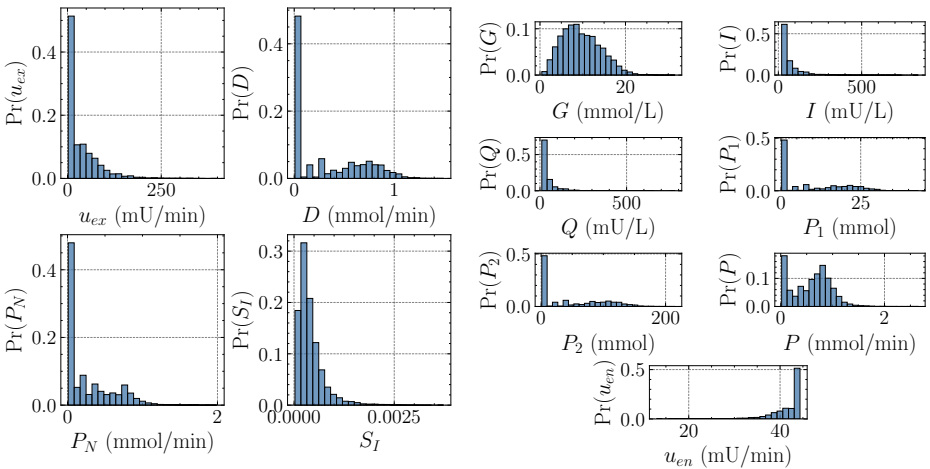


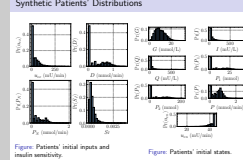
Figure: Patients' initial inputs and insulin sensitivity.

Figure: Patients' initial states.

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 - Synthetic Patients' Distributions



Mention that S_I is taken as a prior from (Davidson et al. 2019)

Estimators Performance in Estimating Blood Glucose

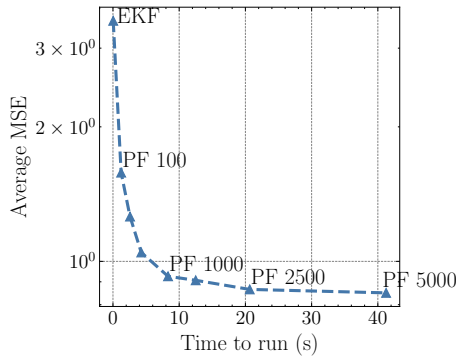


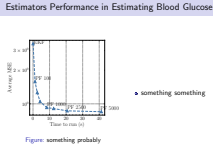
Figure: something probably

● something something

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 - Main Results
 - Estimators Performance in Estimating Blood Glucose



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Specifications:

- 27113 patients in the ICU
- 39.5% of the total number of patients given insulin at some point in their stay
- 100 million minutes of feeding data and 80 million minutes of insulin data to sample from

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└ Results and Contributions

└└ Key Ideas Behind Implementation

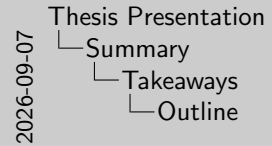
└└└ GLUC-ICU Dataset from Karolinska University Hospital

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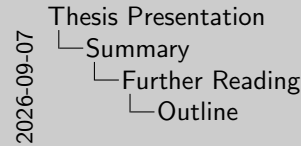
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 [Loneragan, Timothy et al. \(Apr. 2006\). "A Simple Insulin-Nutrition Protocol for Tight Glycemic Control in Critical Illness: Development and Protocol Comparison". In: DOI: 10.1089/dia.2006.8.191.](#)

 [Lotz, Thomas F. et al. \(Mar. 2008\). "Monte Carlo analysis of a new model-based method for insulin sensitivity testing". In: DOI: 10.1016/j.cmpb.2007.03.007.](#)

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